Values and Ethics: high-yield review (about 36% of the exam)

When duties conflict, apply this order

1. Imminent safety
2. Legal mandate
3. Core ethical duties (confidentiality and its limits, consent, self-determination)
4. Assess before acting
5. Start where the client is
6. Least restrictive option
7. Agency policy
8. Worker preference

Confidentiality and its limits

- Explain limits early and as needed (1.07(e)). When feasible, tell the client before disclosing (1.07(d)).
- Disclose without consent only to prevent serious, foreseeable, imminent harm, and only the minimum necessary (1.07(c)).
- **Subpoena is not a court order.** Contact the client and assert privilege; release with consent or a court order. Under an order, ask the court to limit or seal (1.07(j)).
- **Threat to an identifiable person:** assess, then take the protective steps the law requires (Tarasoff, 1976).
- **Child abuse:** reasonable suspicion means you report, yourself. Don't investigate, don't delegate.
- **Adult partner violence:** generally not a mandated report. Assess danger; plan with the client.
- Couples, families, groups: ask members to keep confidences, but say it can't be guaranteed; explain the disclosure policy (1.07(f)-(g)).
- No release, no disclosure, not even confirming attendance. Payers get only what the client authorized (1.07(h)).

Consent and self-determination

- Consent covers purpose, risks, payer limits, costs, alternatives, right to refuse, time frame (1.03(a)). Use qualified interpreters, never children (1.03(b)).
- Lacks capacity: work with the legal surrogate and still inform the client (1.03(c), 1.14). Capacity is decision-specific and can return.
- Mandated clients can refuse; tell them what will be reported (1.03(d)).
- Telehealth: verify identity and location at the start (1.03(f)); the client's location governs.
- **Limits on self-determination:** the Code lets social workers limit it when a client's actions pose a serious, foreseeable and imminent risk (1.02); lack of capacity, legal mandates and minors' legal status also bound it. Otherwise respect unwise choices and plan for safety.

Boundaries, colleagues, records

Issue	Rule
Dual relationship	Avoid if risk of harm; if unavoidable (small town), set boundaries, consult, document (1.06(c))
Sexual contact	Never with clients; former clients: worker bears full burden, no time limit (1.09)
Social media	Decline personal requests from clients (1.06(h)); never post client details (1.07(r))
Gifts, self-disclosure	No dedicated standard: judge by client benefit and conflict of interest (1.06)
Barter	Rare, client-initiated, locally accepted, non-exploitative (1.13(b))
Colleague impairment or misconduct	Talk with the colleague first when feasible, then formal channels (2.08-2.10)
Records	Accurate, timely, relevant; fix errors with dated addenda; label late entries; never backdate (3.04)
Billing	Bill the service actually provided, by who provided it (3.05); no diagnosis to fit coverage (4.04)
Termination	No abandonment; notice and referrals; nonpayment only if terms were clear and no danger (1.17)
Employer policy vs. ethics	Raise and advocate; policy can't override ethics (3.09(d))

Supervising, research, diversity

- Supervisors: stay within competence, no dual roles, fair written criteria, tell clients when a student serves them (3.01-3.03).
- Research: voluntary written consent, right to withdraw, IRB review, support services (5.02).
- Cultural humility: the client is the expert on their culture (1.05(c)). Look for structural barriers before calling anyone "resistant"; advocate (6.04).

Traps to reject

Drastic first moves (reporting to the board, terminating, breaching confidentiality without a basis) · "Consult" when safety action or your own mandated report is due · Agency habit as justification · Family pressure on a competent adult · A professional audience as an excuse to share identifying details.

Sources: NASW Code of Ethics (2021); HIPAA Privacy Rule (45 CFR 164); CAPTA; Tarasoff v. Regents (1976); Reamer (2018).